NCCP 00623 · VERSION 3A

Bevacizumab + capecitabine

Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Mouth

Capecitabine

Painful ulcers or reduced intake: stop tablets and call.

2 Chest

Capecitabine

New chest pain: stop tablets and seek urgent assessment.

3 Bowel / stoma

Capecitabine

Significant diarrhoea, increased output or dehydration needs prompt advice.

4 Hands / feet

Capecitabine

Pain or peeling affecting daily activity: stop tablets and call.

5 Blood / infection

Capecitabine

Fever, shivers or sudden illness: contact oncology immediately.

6 Kidneys / urine protein

Bevacizumab

Urine protein is checked; swelling or reduced urine needs advice.

7 Head / blood pressure

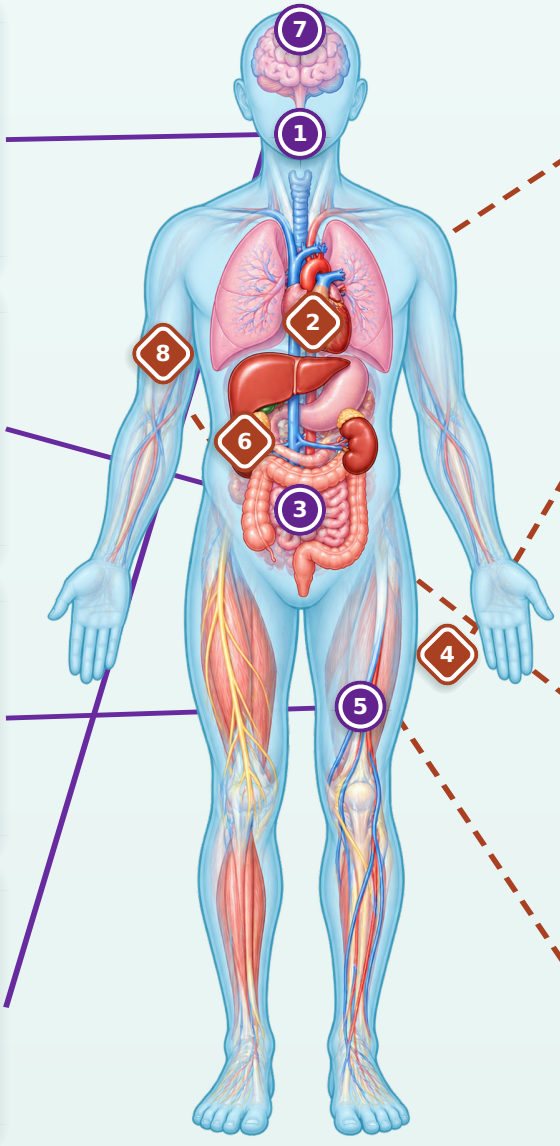
Bevacizumab

Severe headache, visual change or sudden weakness needs urgent help.

8 Bleeding / wounds

Bevacizumab

Report significant bleeding, clots and poor wound healing.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Capecitabine

- Diarrhoea and dehydration
- Painful hands / feet
- Mouth soreness and reduced intake
- Low blood counts / serious infection
- Chest pain / heart problems
- Severe early toxicity, including DPD-related toxicity

Bevacizumab

- High blood pressure / urine protein
- Bleeding or blood clots
- Delayed wound healing
- Rare bowel perforation / fistula
- Rare serious neurological effects
- Infusion reaction

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Nausea prevention and rescue

The team agrees a rescue medicine if needed; routine daily anti-sickness prevention is not automatically required for this oral low-risk schedule.

Bowel symptoms

Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.

Counts and recovery

White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.

Skin and mouth care

Use gentle skin and mouth care. Painful hands and feet or mouth sores can require a tablet hold and team review; do not continue simply to finish the pack.

Blood pressure and wound care

Keep the agreed blood-pressure and urine checks. Report bleeding, poorly healing wounds, new swelling or severe headache promptly.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00623/

Sources and review status are available online. Follow your own prescription and hospital instructions.